The Good Death

If there is such a thing as the good death, it seems like an ideal at best, and an implausible grail at worst. In attempting to identify what such an event constitutes, I will decompose it—as an ideal—into several dimensions: the *medical* good death, the *social* good death, the *political* good death, and finally, the *personal* good death. Considering death as such a richly involved event helps to deepen our understanding of how death can demand just as much, if not often more, from individuals around the dying—whether physicians, other care providers, family, or friends. The examination of the good death also posits considerations, for those of us who are (self-supposedly) less near to death, of how living a good life may function as a prerequisite to the good death.

Sources that portray death largely do so in a fashion aimed at grappling with an aspect of death gone awry—for example, textual representations of the mental and physical torment of a drawn-out dying, the arduously difficult task of securing a burial for a dead parent, and an all-consuming, omnipresent fear of death. Against this backdrop of literary and cinematic death, Pedro Almodóvar's film *The Room Next Door* offers a singular cinematic portrayal of a peaceful, self-determined death, presenting Martha's final moments as a model of the good death. However, the ease of this death seems in part a product of privilege and narrative simplification, leaving many relevant conditions that shape dying unexplored. If we fractionate the death of Martha into medical, social, political, and personal dimensions, and compare these aspects of Martha's death to other portrayals of death, we gain a certain perspective into what the good death demands we think about and aspire to: how to preserve autonomy in an event so humbling and devastating, how to fulfill our responsibilities as both the dying and friends to the dying, and how preparation for this event can make death better, if not good. While Martha's death is modeled as the ideal good death, there are numerous facets to the good death that the film leaves unaddressed—the need for a well-ordered society, for example, and the absence of Martha's daughter in her good death. Martha's death is, in many dimensions, unrealistically removed from the considerations of other texts. Although it acts for us as a model of the good death, in looking at other representations of death, we find that there are additional dimensions necessary to the good death, if such an event exists.

A good death, then, is not a single event but a meeting point of conditions—medical, social, political, and personal. Literary representations of death suggest in tandem that when a number of these dimensions fail in some capacity, the possibility of a good death collapses. While death is often conceived as either largely medical or deeply personal, literary and cinematic portrayals posit that a "good death" depends on a fragile convergence of medical care, social relationships, political structures, and personal preparation. In comparing the controlled death of Martha in *The Room Next Door* to more troubled deaths in works such as Leo Tolstoy's *The Death of Ivan Ilyich*, Margaret Edson's *W;t*, and Alfonso Cuarón's *Children of Men*, we see that the good death is less an individual achievement, and more broadly a collective and structural ideal. This grows even more clear when we compare the role of Ingrid in Martha's good death to the family of the dead or dying in Khaled Khalifa's *Death is Hard Work*, Simone de Beauvoir's *A Very Easy Death*, and Rebecca Brown's *Gifts of the Body*. Our evaluation of the good death must additionally consider how our "good life" may come at the expense of others' good lives and good deaths, as in Kazuo Ishiguro's *Never Let Me Go*. With these works in mind, I will explore the medical, social, and political dimensions of death, what makes Martha's death "good" with respect to these dimensions, and how other works can help us better understand dimensions left unaddressed by the film. As we consider the good death and its numerous facets, *The Room Next Door* appears more and more idealistic, suggesting that the good death is an ideal we should strive towards but may, in the end, still find unattainable.

The Medical Death

The medicalized death, as a relatively recent phenomenon frequent to the U.S., takes place most often in a hospital, requiring that physicians and health care providers play the most significant role in mediating the good death. What this means, however, is that death feels much less proximate to everyday life and those outside the domain of medicine. This shapes death into an event where the responsibility of securing a "good death" falls most heavily to healthcare professionals. Modern medicine has concentrated authority over death in the hands of physicians, yet texts like *The Death of Ivan Ilyich*, *A Very Easy Death*, and *W;t* repeatedly argue that this authority often obfuscates the good death. Physicians are characterized as avoidant of honest confrontation with death and pursuant of treatment to extend the patient's life over comfort in their final days, thereby alienating both themselves and their patients from the process of dying

and the attainment of the good death. The good medical death, it seems, requires physicians to play a role built not on their medical expertise, but on their capacity to provide care grounded in humility, honesty, and an understanding of what the patient's good death looks like to the patient and their loved ones. Medicine, as these texts show, has the capacity to extend one's lifespan, but subsequently to disaffect the dying person from thinking about and preparing for their good death.

The power and authority accorded to physicians by their specialized knowledge and decision-making drives a sense of power imbalance, producing consistent criticisms across textual portrayals of physicians. This doctor-patient power dynamic parallels that of a judge-defendant in Leo Tolstoy's *The Death of Ivan Ilyich*, and that of a professor-student in Margaret Edson's *W;t*. The same authority that Ivan Ilyich enjoys wielding over others as a judge comes back to humble him in his illness: "this is all nonsense, an empty sham, but when the doctor…with the gravest look on his face performs a selection of gymnastic contortions, Ivan Ilyich accedes to this just as he once acceded to speeches from lawyers even when he was well aware that they were lying, and why they were lying" (203). Beauvoir, in *A Very Easy Death*, uses a similar legal analogy for the physician-patient power dynamic: "Dr J, Professor B, Dr T … I recognized that piddling self-importance: it was that of the judges on the bench when they have a man whose life is at stake before them" (21). The asymmetry in power and reversal of roles is even more explicit in *W;t*, as Vivian observes, "in Grand Rounds, *they* read *me* like a book. Once I did the teaching, now I am taught" (37). Beyond the inevitable humbling brought by proximate death, the role reversal between Vivian and her former student, Jason, gives way to strong criticisms of the young doctor. Jason exemplifies the worst criticisms of physicians, from his leaving Vivian, still in stirrups, in the middle of a pelvic exam—"having a former student give me a pelvic exam was thoroughly *degrading*" (32)—to his complete lack of regard for provision of quality care, deeming bedside manner a "colossal waste of time for researchers" like him (55). However, Vivian sees herself in Jason, not only because of the pronounced reversal in power, but in a sort of generational inheritance of values: "[t]he young doctor, like the senior scholar, prefers research to humanity. At the same time the senior scholar, in her pathetic state as a simpering victim, wishes the young doctor would take more interest in personal contact" (58). Interestingly, in tandem to the role-reversals of Ivan Ilyich and Vivian Bearing, their most highly regarded principles and values are also upended. For Ivan Ilyich, the sense of propriety that

guided his every life decision drives a tumultuous confrontation with a deep-seated dissatisfaction with the pretenses of his life. As she approaches the end of treatment, and more importantly the end of her life, Vivian, too, abandons John Donne and the "outstanding human faculty of the era, namely wit" (48)—both of to which she dedicated her career—"we are discussing life and death, and not in the abstract, either; we are discussing *my* life and *my* death… Now is not the time… for wit… Now is a time for simplicity. Now is a time for, dare I say it, kindness" (69). In an event such as the medical death where the dying are so deeply humbled, the judge now the judged and the teacher now the taught—"I thought being extremely smart would take care of it… I'm like a student and this is the final exam and I don't know what to put down because I don't understand the question and I'm *running out of time*" (70)—the role of the physician as a figure of authority becomes particularly tenuous.

The texts additionally share a criticism of physicians' inability to confront patient death, which subsequently prevents them from helping the patient prepare for death. A great torment for Ivan Ilyich at the end of his life is the physicians' inability to look death in the face: "Is this condition life-threatening or not? But the doctor treated this question as irrelevant, and ignored it… the only thing was a balancing of probabilities… this conflict between the floating kidney and the blind gut"(183). Tolstoy's criticism here is extended to everyone around Ivan Ilyich, but is especially pertinent to the medical death in considering the role of the physician. While such a characterization of physicians is less explicit in *W;t*, it is still prominent—"Here, 'Rounds' seems to signify darting around the main issue… which I suppose would be the struggle for life" (36). Indeed, Kelekian and Jason do little to prepare Vivian for death, seemingly capitalizing, instead, on her determination to be distinguished, even in illness—to be the first to make it through the full eight cycles of the clinical trial: "I wish they could all get through it full throttle," Jason remarks (75). The inability of these physicians to forefront the nearness of death to the patient makes it all the more difficult for the patients themselves to prepare for death, let alone consider how to achieve a good death. Additionally, the physicians seem to lack an understanding of what matters most to their patients; for example, sympathy and comfort for Ivan Ilyich (200). Not only do they have little sense of what would make the dying and death for their patients easier, compounded by their unwillingness to confront death, they seem to have little desire to make sense of what matters to their patient's good death. Kelekian, too, is apathetic in his provision of care, saying to Vivian after a cycle of the clinical trial destroys her immune system: "Isolation is

no problem. Couple of days. Think of it as a vacation" (46). This parallels a quip by Martha to Ingrid—"just think we're on vacation" (56:00)—from early on in their preparation for Martha's good death, and one that is taken poorly by Ingrid. A large part of physician apathy towards the patient's values may very well be due to their inability to speak directly and honestly about death to the patients; the other part might serve a diagnosis of how difficult it is to provide the dying the ideal sort of end-of-life care.

If these criticisms are not enough, Edson and de Beauvoir both describe physicians with agendas that prioritize research, a proxy of medical prestige, above patient wellbeing. Nurse Susie, referring to Jason and Kelekian, says "Well, they like to save lives. So anything's okay, as long as life continues" (68). However, this comes at the cost of the good death, or at least the possibility of one, for Vivian, who remarks, "[m]y treatment imperils my health" (47). Even Vivian's wishes, in death, are disrespected by Jason—when Susie says, "She's DNR!" he responds, "She's Research!" (82). De Beauvoir, too, makes similar observations in her ever-perceptive assessment of those caring for her Maman: "Dr N and I did not get along together at all… for him she was the subject of an interesting experiment and not a person. He frightened us" (51-52). While we can take these physicians, perhaps, as warnings of how the doctor should not contribute to the good death, there is clearly more to be done, as Gawande acutely senses, in readying physicians for their responsibility in helping patients prepare for the good death.

Though these textual representations grant physicians little grace, other healthcare providers are given more. Whether this is due to the institutional power differential, or a byproduct of the different demands on healthcare providers like nurses and caretakers, these representations portray kindly and optimistically the contribution of such healthcare professionals to the good death. An obvious foil to Jason, the nurse Susie in *W;t*, is the only character who considers Vivian's comfort, rubbing her hands with baby oil (78), listening to Vivian (71), and soothing her, as if she were a child (65). De Beauvoir again agrees with *W;t*'s characterization of nurses in her own evaluations: "I did like the nurses; they were linked to their patient by the extreme closeness of those necessary tasks that were humiliating for her and revolting for them, and the interest they took in Maman did at least have the appearance of friendship. Mlle Laurent, the physiotherapist… never assumed any superiority" (22). The intimacy of the care provided by these other healthcare professionals, a number of which notably

occupy engendered roles, seems to create room for them to contribute something better to the medical good death. The marked difference between how these works treat physicians and nurses seems to stem from differences in the nature and demands of their occupations.

These works thus depict physicians not as guides toward a good death but as figures whose professional demands—diagnosis, research, and clinical control—distance both themselves and their patients from an honest confrontation with mortality. The criticisms of physicians shared by these texts confirm the perduring fact that physicians are very human. They take part in the same "gymnastic contortions" as the rest of us, and it concomitantly seems a great ask to have physicians provide each patient the level of care Rebecca Brown does in her *Gifts of the Body*. The diagnosis of physicians viewing the end-of-life body as a biomechanical problem is indeed too shallow. Instead, I now wonder—is it sustainable or even realistic for physicians to assume so much responsibility for the good death? We see the toll of a similar responsibility on healthcare providers for AIDS patients in Brown's work; the care providers require "respite" from caring for their dying person. While the physicians we have encountered, both real and fictional, have been criticized for their lack of contribution to the good death, what *is* their ideal role in the good death? Atul Gawande, for one, boils this down to a matter of asking patients what matters most to them. Certainly, honesty and patient-centric bedside manner are critical to this discussion— and Tolstoy and Edson would seem to agree with this call for a shift in medical care decision- making from paternalistic to shared—but healthcare professionals are entirely absent from Martha's good death in *The Room Next Door*. If a physician had been present, or even aware of Martha's preparations for her own death, the film would have gone very differently; we might not be able to call Martha's death a good death. The film is similar to other works in its presentation of physicians in that the wishes of Martha's doctors are contrary to her own, and parallels emerge to Vivian Bearing's own experiences in *W;t*. Physicians have clinical trials, research, and prestige that sway the care they provide to patients. Their profession also rests, necessarily so, on a faith in medicine that binds them to these sorts of predilections. It would be ludicrous to ask physicians to doubt medicine, even so as to re-consider how alternative treatments (or a lack thereof) could contribute to a patient's better death. The idea that medicine can stave off, or even control death is foundational to the practice of medicine and has only increased with contemporary proliferation of machines and medicines; to instead think of

medicine as a means of helping people shape their dying seems askew of the physician's professional mission.

The Room Next Door seems to suggest the physician has no role to play in the good death, or that the good death proceeds best without any contribution from the physician. This, perhaps, is because we can imagine that Martha's doctors would have neither considered Martha's death a good death, nor contributed to her conceived good death. The issue then emerges that the good death for the dying is not always "good" for those around the dying. Are we then meant to define the good death solely from the perspective of the dying? *The Room Next Door* still posits otherwise, as Martha's good death seems more of a negotiation between the good death considering both Martha and Ingrid; we see thought given, throughout the film, to how Martha's good death will affect Ingrid. The ideal of Martha's death, if transposed to the medical good death, presents the good death as an event that must be mediated by considerations of and from everyone involved. Unlike Jason in *W;t*, physicians must consider the good death with respect to the wishes of the patient, and there is a conversation to be had where the physician does this alongside the patient and those around the patient. These texts agree that there is something lacking in the way physicians help prepare their patients for the good death. Gawande's championing question in *Being Mortal*—what matters most to you?—is certainly an apt start, but there is more to be said, especially with regard to the deep torment Ivan Ilyich feels on his deathbed, and in Vivian's final realization of the necessity of simplicity and kindness. Martha's good death, outside the authority of physicians, suggests the good death may be easier to achieve outside medical authority. The absence of medical supervision allows Martha to shape the conditions of her own death according to her own values. While the physicians in *The Room Next Door* might view Martha's actions as some sort of failure of medicine, the narrative presents them as a fulfillment of autonomy. However, it is clear that Martha's procurement of a euthanasia pill grants her a reprieve from how the medical death commonly proceeds. Despite the absence of institutionalized medicine in Martha's good death, to realistically consider the good death maintains that there is certainly a role for physicians to play in the good medical death—one that demands more honest confrontation of death, an understanding of what matters to the patient's good death, and a re-positioning of both considerations above personal agendas of prestige, research, and superiority.

The Social Death

> Death is a solitary experience, of course, but nevertheless it lays heavy obligations on the living… The living have a harder task ahead of them than the dead (16).

Early in the novel, Khalifa's *Death is Hard Work* is already inlaid with weighty observations about the social death. Thinking about the social death encourages us to consider how death affects those other than the dying: what these "heavy obligations" look like, how we are meant to oblige, and how our obliging contributes to their good death. Although the development of life-extending technologies has shifted death from the home to the hospital, family and our broader social communities still clearly have a role to play in contributing to the good death. The social good death can be considered to involve those around the dying in two senses: (1) the inner circle of family and friends to the dying, and (2) a broader social community, extending as far as modern society and culture. *The Room Next Door* takes a strong position on the requisite of being accompanied in the good death. Interestingly, this is fulfilled by Ingrid, an old friend, and not Martha's daughter, who only appears in the film towards the end. The role played by Ingrid in Martha's good death, nonetheless, is critical to our understanding of how friends to the dying can contribute to their good death. As the role of family is not explored by the film, we turn instead back to Tolstoy's *The Death of Ivan Ilyich* and de Beauvoir's *A Very Easy Death* to evaluate the responsibilities of family and friends to the dying (or dead). For the contributions to the good death by a broader social community, we then look to Ishiguro's *Never Let Me Go*.

For Ivan Ilyich's family, Tolstoy's text serves first and foremost as an indictment: "'What have we done wrong? Liza asked her mother… 'I'm sorry for Papa, but why do we have to suffer?'" (213). In part, their failure to contribute to Ivan Ilyich's good death lies in their continued preoccupation with propriety and lived pretense, thinking only of themselves and how this suffering disrupts their agreeable, comfortable charades of decency. Only Ivan Ilyich's son and his servant, Gerasim, appear exempt from these criticisms: "This was why Ivan Ilyich felt comfortable only with Gerasim… Gerasim was the only one who didn't lie to him… 'We've all got to die one day. Why shouldn't I give you a hand?'" (199-200). Tolstoy's criticisms more broadly criticize an aspect of the human condition, but we can accordingly take away a narrower sense that family to the dying should aspire to provide authenticity and honest companionship.

Even for a figure so tall as Simone de Beauvoir, the obligations she bears to her mother in her dying and death take a heavy toll. While it may have been an easy death for her Maman, it certainly is not for de Beauvoir: "I did not particularly want to see Maman again before her death; but I could not bear the idea that she should not see me again. Why attribute such importance to a moment since there would be no memory? There would not be any atonement either" (62). Indeed, de Beauvoir attributed great importance to "the last moments of a dying person," where "the absolute could be enclosed" (63). Simone de Beauvoir's somewhat puzzling final lines to *A Very Easy Death* posit that there is no such thing as the good death: "There is no such thing as a natural death… for every man his death is an accident and, even if he knows it and consents to it, an unjustifiable violation" (106). If the good death is possible, it certainly requires some sort of preparation, but de Beauvoir seems to think that even this is inadequate to make death better. Her memoir also suggests that, although, "indeed, comparatively speaking, [Maman's] death was an easy one… [s]he had a very easy death; an upper-class death," the easy death is far from the good death (94-95). Whether this is because death itself is, as she describes it, "an unjustifiable violation," or if she still feels conflicted over her own role in her Maman's death, we nevertheless get the sense that death is an event no amount of preparation can make good. Brown, too, observes, "[t]here was always a hole when someone died. It was always in the middle of people" (157). The good death in these texts indeed seems near impossible to achieve, especially for the living who are left to face the deeply acute loss of a cared one.

This sense of violating loss may be an inescapable experience inherent to death as an event, but both Brown's morally praiseworthy care in *Gifts of the Body* and Almodóvar's *The Room Next Door* suggest there are steps we can take to make a better death possible. De Beauvoir herself seems unsure about the role she played in her mother's death, and whether the choice to conceal from her mother her dying is the right one; however, we might surmise that her mother must have had some sense she was at the end of her life. De Beauvoir's choice to not tell her mother she is dying, in consideration of how to make the good death more possible for her Maman, may have been all she could have contributed in her role as family to the dying; the concealment, after all, certainly was not for her own peace of mind—"she was going to die. She did not know it: but I did. In her name, I revolted against it" (83). When put in conversation with the other works we have encountered, *A Very Easy Death* also complicates the need for authenticity in the good death. However, Simone de Beauvoir's deception of her dying mother in

A Very Easy Death feels very distinct from the lack of authenticity, for example, from Ivan Ilyich's family members as he approaches death. Ivan's wife and daughter refuse to confront his death, more because they continue to live their lives under a facade of propriety. De Beauvoir, on the other hand, contradicts the principle of authenticity so central to her works, such as *When Things of the Spirit Come First* to *The Ethics of Ambiguity*, in order to provide her mother what she believes might be an easier death, though in the end, perhaps not a good one (in the eyes of de Beauvoir, at least). This is all to de Beauvoir's own great tumult—"you feel that she should have had more room in your life… you never do all you might for anyone… you have plenty of room left for self-reproach… We felt that we atoned for this by the days that we gave up to her, by the peace that our being there gave up to her, by the victories gained over fear and pain" (94). The ambivalence in *A Very Easy Death* starkly illustrates our human limitations: as we navigate death as an event, we cannot step outside of circumstances like a complicated family history. It is extremely difficult to agree or disagree with de Beauvoir's own role in her mother's dying; we are left, alongside her, frustrated with the event of death, and that we have to choose a side at all.

Martha's daughter does not even appear in *The Room Next Door* until after Martha dies, yet her absence from Martha's good death is not particularly pronounced. Instead, Martha's relationship with Ingrid becomes the focal point of Martha's death along its social dimension. At the opening of the film, Martha and Ingrid are not particularly close, and Martha's request for Ingrid to be in the room next door when she dies feels somewhat inapposite. What the film then proceeds to portray singularly is the negotiation between the two as they both prepare for Martha's death. From Martha offering Ingrid a sedative to calm her (58:08), to the leaving of the door closed as a "sign" that Martha has passed (55:31), and even in deliberation of the legal aftermath (54:59), the good death is an event that is made good not just by Ingrid being there for Martha, but also by Martha's consideration of how her dying affects Ingrid. This model suggests that in addition to companionship from friends of the dying, there are steps the dying should take to ease the burden of their death on the living. Nevertheless, it still feels conspicuously odd when Martha's daughter graces the screen at the close of the film, even lying where her mother passed away, eyes closed. In this regard, perhaps de Beauvoir's narration of her mother's dying, with its blunt account of their complicated family history, is a more realistic representation of the role of family in the social good death.

Extending the scope of the social good death outwards, while intimate social relationships in *The Death of Ivan Ilyich* and *A Very Easy Death* provide valuable insight into how, as individuals, we should accompany the dying, Ishiguro's *Never Let Me Go* widens the lens to modern culture and society. Ishiguro constructs a world where an entire social class comes into existence because of the fear of death. In this society, death is displaced onto others as organ donors; society accepts the deaths of these organ donors to prolong the lives of others. Society in this novel surely fails to contribute to the good death. However, Madame and Ms. Emily certainly seem to attempt more with Hailsham and the Gallery: "we at least saw to it that all of you in our care, you grew up in wonderful surroundings… [w]e were virtually attempting to square the circle… there would always be a barrier against seeing you as properly human. Well, we fought that battle for many years, and what we won for you, at least, were many improvements, though of course, you were only a select few" (261-263). In some feeble but not insubstantial way, they do make a contribution to a better life for Kathy, and perhaps subsequently a better opportunity for a good death. After all, despite being created for organ donation—their lives structured entirely around societal fear of death—Kathy, Ruth, and Tommy still form meaningful relationships and formative memories. The role of medicine in extending life returns in this societal fear, abused along another dimension. Interestingly, *Never Let Me Go* disassembles the platitude that love conquers all evil, but concomitantly suggests that even in an unjust system, social companionship betters the lived experiences of life, and perhaps subsequently death. Kathy's memories of Hailsham, her relationships with Ruth and Tommy, and her several years as a carer suggest that Kathy has, at the very least, a better death than she would have without said social connections and interactions, if not a good one. Whether Madame and Ms. Emily contributed adequately to that better death is unclear; we may have to accept that, at some level, that they contributed at all was enough.

The Political Death

If the medical and social dimensions of death concern the more immediate circumstances of dying and death, the political dimension of the good death concerns the conditions that make the good death possible in the first place, as well as the bureaucratic necessities after death. Political instability or injustice seem to make the good death structurally impossible; when political institutions collapse or become oppressive, even the most basic rituals (e.g., burial)

become difficult or impossible to fulfill. It thus follows that the government and a well-ordered society have a certain role to assume in making the good death possible. Khalifa's *Death is Hard Work* and Cuarón's *Children of Men* broach this issue from different angles, one set against the Syrian civil war's landscape of bombed highways and manned checkpoints, and the other in a not-so-dystopian world of forced deportation of "fugee" immigrants, cages and detention camps, and an aging population, inching toward extinction. *Death is Hard Work* foregrounds a fractured landscape of regime-operated checkpoints, revolutionaries, and extremists amidst which living is hard and securing the good death is even harder. *Children of Men* has the added consideration of how the good death changes in a world of mass infertility. The political dimension of death is particularly important to think about because death on a large scale occurs more often than we think or see, and is perhaps where the good death is the most out of reach.

The political landscape so central to *Death is Hard Work* lays waste to many of the customary conventions of death: "Rites and rituals mean nothing now… Death wasn't even a source of distress anymore: it had become an escape much envied by the living" (5). While life for the living seems to diminish in value, so too do the remains of the dead—the corpse of Bolbol, Hussein, and Fatima's father, as they begin to transport it per his dying wish, becomes "a commodity like hookah coals, crates of tomatoes, sacks of onions" (17). As Abdel Latif's corpse proceeds to rot, crawl with maggots, and attract wild dogs, we find ourselves wondering why Bolbol, Hussein, and Fatima are still even trying to fulfill their father's request. Certainly, even early in this journey, we see their regret for having even started: "[Bolbol] wished his father's body would turn into a sack of cumin; it was hard to see any downside to such a transformation—in fact, reaching a state of mutual understanding with a sack of cumin would be easier and far less dangerous" (38). While *Death is Hard Work* demonstrates that even a ritual like burial, so taken for granted in death as an event, becomes near-impossible in such a war-torn setting, why was it so important that the father's last request be honored? If Bolbol, Hussein, and Fatima had left their father's corpse to be eaten by wolves, would Abdel Latif's death have been less good? After all—he was already dead. Even before his children escort his corpse to Anabiya against all odds, Abdel Latif, in many ways, seems to have a good death—a remarkably good death in the Syrian civil war zone. He dies in old age, having "had done everything he wanted to do in this life and [having] … said everything he needed to say" (39). His death is very hard work—not for him, but for his children, one of whom is left uncharacteristically reserved,

another who changes his name, and the last of whom goes permanently mute. At the close of the novel, death feels like very hard work indeed, and Abdel Latif al-Salim's death, while good for him as the dying, left a terrible aftermath for his family. This, of course, is in large part because of the political landscape, but also encourages us to think about how much the dying should consider the effects of their death on their loved ones.

There is a converging sense from both works that, despite being closer to death as a society, life has somehow become less meaningful. In *Children of Men*, the government issues Quietus kits for citizens to "quietly" end their own lives, but it feels less like Martha's dark web euthanasia pill—a means of autonomy in death—and more like acquiescent disregard, even offering "[u]p to £2,000 to your next of kin" as a kit benefit (11:30). When we are first introduced to Theo's character, we get an immediate sense that he sees little value in his own life: "At least with a hangover I feel something. Honestly, Jasper, sometimes…" (07:04). The society of this world prizes youth over life itself, where youth, perhaps, functions in this world as a proxy for childhood; the baby's cry is what brings the fighting between the Fishes and the government to a truce, if only momentarily. Of the many deaths in the film, Theo's seems the closest to a good death, as he dies with the knowledge that a child has been born and that humankind will not go extinct; his life seems to gain additional meaning from this, and his death subsequently becomes "better." The child being named "Dylan" cements the sense that in some ways, Theo is the father of Kee's infant. Mass infertility, in concept, does not affect the deaths of those living, but it nevertheless strips away meaning from their lives and subsequently, their deaths. Marks of humanity, like art (as in *Never Let Me Go*), become meaningless, and we are left to ponder how, as time becomes more finite, we should use that time.

In *The Room Next Door*, the only gesture to this political dimension of the good death is in Damian's stance on not having children considering the pressing climate crisis (1:20:44). He is the only one who seems to think about life and death in this more realistic regard, but it comprises a slim few minutes of the film. What we can take away is the sense that death is rarely as controlled and removed as Martha's is; her good death is in part good because she does not need to consider this political dimension of death, which pushes the ideal further out of reach. The good death therefore cannot exist in isolation from its political dimension, though *The Room Next Door* leaves it poorly explored. When societies are fractured by war, oppression, or existential despair, the medical and social practices that support the good death collapse

alongside them. This political dimension pushes us to think more broadly about death as an event that concerns all of humanity rather than just ourselves and those we know. It thus becomes important to recognize that our ability to approximate the good death is deeply affected by political and sociocultural moments, and there is a more public responsibility to be assumed for these dimensions of death.

The Personal Death

What steps should the dying take to assure the good death? Is there such a thing as the good death, and if so, how should we describe it? Should we fear death, just dying, or both? Why or why not? These questions shift our focus of the good death away from relationships and institutions toward the individual's own confrontation with mortality. The good death seems to largely depend on the individual's ability to integrate mortality into our understanding of life. There is a balance to be struck between being aware of death and living in constant fear of death. In thinking on the personal good death, there also seems to be a fuzzy relationship between the good death and a good life, or a life well lived. Does a life, unsatisfactorily lived, preclude an individual from having a good death? How often should we think about our death; when we do think about our death, how should we do so; and how should these considerations affect, or not affect, how we go about our lives? Certainly, a life governed by the fear of death seems like a poor way to live. On the other hand, a life with no thought given to death excludes an individual from the opportunity to think about and prepare for their good death. There is a very personal dimension to death, revolving around the dying individual, that ties together the medical, social, and political dimensions that we have already considered in the good death.

For Ivan Ilyich, "[w]hat was preventing him from getting through was his insistence that his life has been a good one" (215-216). Whether Ivan's final moments are effectively redeeming for him, the drawn-out torment that makes his dying so unbearable stems from his unwillingness to reconcile himself with "the living lie demanded by propriety;" a refusal to confront the deep-seated sense that perhaps he had not lived his life as he should have (207). In some ways, Ivan's dying is his only attempt at reconciliation—if he had never been forced into this realization, would his death have been better? The text seems to think otherwise; the death of Ivan Ilyich seems inextricably tied to the life he led. His death is full of anguish as he questions the life he lived, but sparing him of this confrontation would not have made his life better, and

therefore might not have made his death better either. For Ivan, it seems that the life he lived obstructed him from having a good death. It is only in his final moments that we see any indication otherwise: "He opened his eyes and looked at his son. He felt sorry for him… 'Must *do* something… Take him away… sorry for him… and you…' He tried to say, 'Forgive me'" (216). Whether his final self-reckoning made his death better (or at least less "bad"), the death of Ivan Ilyich encourages us to think about the meaning of the life we lead, and suggests that this might in itself prepare us for a better, if not good, death.

In *Never Let Me Go*, death is oblique and difficult to assess. Given that we are reading, from "outside the whale," to use George Orwell's metaphor, it is difficult to imagine that Kathy's death would have been a good one, especially considering her complete lack of autonomy, a value so central to Martha's good death in *The Room Next Door*. However, stepping into the whale with Kathy, her death was relatively quite good; Hailsham, in its clumsy attempt to gild the donors' dismal fate, provided them a childhood with companionship—"that was when I first understood, really understood, just how lucky we'd been—Tommy, Ruth, me, all the rest of us" (6). In the novel's society, Hailsham offers one of the only attempts at change, moving beyond everyone else's curious resignation to fate. Moments of outrage and reckoning appear throughout the book in mentions of other Hailsham-esque institutes and in the figure of Miss Lucy, but the novel ultimately reads like a casual retelling of a tale of friendship rather than an account of the horrors of the characters' fates. The gentle, placid prose and calm, inoffensive diction only serve to make *Never Let Me Go* all the more unsettling: "[T]he idea of things 'unzipping'… [became] a running joke among us about the donations. The idea was that when the time came, you'd be able just to unzip a bit of yourself, a kidney or something would slide out, and you'd hand it over. It wasn't something we found so funny in itself; it was more a way of putting each other off our food" (87-88). The novel is concerned with neither the science required for cloning and organ transplantation, nor the ethical ramifications or political factions behind these procedures. Instead, we are left to consider that, despite inclinations otherwise, Kathy's life had more meaning because of Hailsham. It is easy to imagine that, without Hailsham, her death could have been much worse, but it still feels spurious to consider her death as good. Broadly, Ishiguro seems to be encouraging us to consider that there is a cost to the way we live that we do not necessarily see and rarely acknowledge; this concerns not only our own matters of life and death,

but the lives and deaths of others. Grimly taken, this commentary on the human condition echoes a line from Blaise Pascal's *Pensées*:

> Imagine a number of men in chains, all under sentence of death, some of whom are each day butchered in the sight of the others; those remaining see their own condition in that of their fellows, and looking at each other with grief and despair await their turn. This is an image of the human condition.

Just as unsettlingly, the Hailsham staff feel "such revulsion" by the donors, even in their efforts to prove to society that they are human (269). In the end, Kathy and the others are the most privileged in an exploited class of humans, and we are left to consider how we are complicit in the exploitation of others' suffering, and perhaps in the exploitation of our own lives: "when I watched you… I saw a new world coming rapidly. More scientific, efficient, yes. More cures for the old sicknesses. Very good. But a harsh, cruel world. And I saw a little girl, her eyes tightly closed, holding to her breast the old kind world, one that she knew in her heart could not remain, and she was holding it and pleading, never to let her go" (272). The novel's title is a very human, very impossible request; there is nothing we can hold on to forever. Does death—our finite existence—give our lives meaning? As compressed as the donors' lives are, Kathy manages to achieve something with hers, finding meaning in her twelve years of being a carer. In many ways, the novel is uncannily realistic for a text that is often read as soft science-fiction: we, too, are born to die; "[i]n the end, we can't stay together forever" (282). There is nevertheless room for us to achieve meaning in our lives, concomitantly preparing ourselves for a better death.

As preparation for the good death requires us, of course, to think about death, the fear of death naturally emerges—not just as a byproduct of preparing for death, but sometimes as an obstacle to our preparation for death. The suburban heterotopia that Don Delillo's *White Noise* is set in does little to mute the disquieting fear of death that is so formative for Jack's character. In fact, the novel can be read in such a way where Jack's every action can be interpreted as decisions driven by his fear of death. From his academic dress, "[i]t makes you feel like a stranger in your own dying. I wanted my academic gown and dark glasses" (142), to his discipline of Hitler Studies, "Hitler is larger than death. You thought he would protect you. I understand completely" (287), to the naming of his son, "I thought it was a forceful name, a strong name. It has a kind of authority… I wanted to do something German… to shield him,

make him unafraid" (63)—Jack's every move is one that attempts to hide or shield himself from death. In an interesting parallel, Vivian in *W;t* recounts an encounter with her student: "I think it's like he's hiding… maybe he's scared, so he hides behind all this complicated stuff, hides behind this *wit*" (60). In this encounter, she quickly rebuffs the student, but their remarks seem to bear some truth—"[m]y only defense is the acquisition of vocabulary" (44)—and is conspicuously similar to Jack's "shield" of Hitler Studies in *White Noise*. Of course, neither Hitler Studies nor John Donne prepare them for the good death. Vivian does not seem to fear death as ubiquitously as Jack, but both characters serve as warnings that no amount of intellectualizing can loosen the hold of death as an event, and that the fear of death itself can draw us further from the good death. We must think about death, but we must think about it healthily; we cannot think about death all the time, and we certainly cannot let fear of death dictate the kind of life we lead. There is a very human quality to such fear of death, but also something dangerously enthralling, something *White Noise* warns us of.

Though we bear in mind these warnings against the fear of death, a large part of the good death for an individual seems contingent on the fact that they do imagine their good death, and how exactly they imagine their good death. What matters most when one thinks about the good death? Indeed, it seems rare to have such clarity, as Martha does, towards what one wants at the end of their life. She articulates a very clear picture of how she wants to die, and she has the means to make it a reality: "You can't be self-possessed if you're in agony…you can only think of one thing… I think I deserve a good death… going out with a little bit of dignity, clean and dry" (27:43-28:33). Her re-framing of terminal illness opposes the coercive language of fighting cancer—"Cancer can't get me if I get me first"—and expresses her determination to preserve control over her death. Martha's clarity regarding death in *The Room Next Door*, while perhaps not realistic, suggests that the good death may depend less on eliminating fear, and more on confronting our mortality honestly. If Ivan Ilyich suffers because he realizes too late that his life may have been wrongly lived, and Jack Gladney and Vivian Bearing hide behind intellectual defenses against death, Martha instead acknowledges death directly and prepares for it according to her own values. The personal good death, then, may lie not in avoiding death or trying to master it entirely, but in approaching it with a degree of honesty, preparation, and acceptance.

Returning to the Good Death

The good death, in the end, belongs not only to the dying, but also to the living who accompany them at the end of their life. Piecing together the dimensions necessary to the good death as an ideal, just as a very easy death is not necessarily a good death, a good death is rarely easy. In the medical dimension of death, I agree with Gawande that there is a deep sense that some radical change must occur in the way physicians think about their patients' deaths, and how they then think about death alongside their patients. Along the social dimension of death, family, friends, and broader social communities peripheral to the dying all have a role to play in care and companionship, and death as an event inevitably takes a significant toll on the living. There are steps the dying can take to make their death more bearable for those around them; an ideal reciprocity is displayed by Martha and Ingrid in *The Room Next Door*. However, the roles played by healthcare professionals, and family and friends to the dying, largely collapse when the political dimension of the good death breaks down. Political instability makes the good death very different, and very difficult to achieve. It pushes us to consider death on a larger scale, and how, when conventions of death taken for granted fall apart, the good death becomes harder and harder to achieve. Finally, and perhaps most important to the good death, the dying have much to do in thinking about what matters most to them in their good death, and negotiating their good death with the medical professionals and loved ones around them. For those of us (who think we are) not yet at the end of our lives, there is still additional work to be done in navigating awareness of death, book-ended by denial as one extreme, and despair as the other. Martha is a good, if idealized, example of confronting our mortality with honesty.

The political dimension of the good death, in many ways, is not one we can control as we prepare for the good death. However, it is certainly one worth thinking about as the backdrop to many of the medical, social, and personal considerations that are more obvious to death as an event. Part of what makes Martha's death so unrealistic is that it feels removed from the political dimension of death; unconcerned with the issues only Damian confronts in the film. The political good death deeply affects elements of the medical, social, and personal dimensions of death—in *Death is Hard Work*, we see it shape how people, including medical professionals, treat death as an event; the family dynamics of Bolbol, Hussein, Fatima, and their late father; and what constitutes a good personal death, including rituals like burial. The medical and social dimensions of death are similar in that they demand something(s) of both the living and the

dying, converging in a need for authenticity and understanding. The personal death evaluates more deeply the private obstacles that stand in the way of a good death, from how an individual lives their life to whether they think about death with denial or despair. The good death accordingly hinges on these dimensions of death; we may not need to fulfill all of them, but the collapse of any one greatly hinders the possibility of a good death.

As we look to *The Room Next Door* as an ideal of good death, it accordingly appears that the good death may not be so attainable to most of us. It certainly feels deluging to think that we should fulfill every dimension of death—medical, social, political, personal, and even others not addressed here—but they can serve as benchmarks, perhaps, for thinking about ways we can negotiate death into a more bearable event. In striving for the good death, we can approach a better death, for the sake of both the dying and those by their side. Whether the good death is possible, it certainly exists in ideation, and can thus guide the way we live our lives, accompany the dying, and prepare for our own inevitable deaths.

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Acknowledgements:

Deep thanks to Elinor VanGilder, for extensive feedback on argumentation, intertextuality, and more; to Julie Wong, for comments on clarity and wording; to Emma Quirk, for letting me watch and re-watch *The Room Next Door* on her Netflix account.